

Psy408 Health Psychology Questions Answers

Q1. How to manage epileptic behavior if someone has a seizure in front of you

- Stay **calm** and ensure the person is safe.
- Move any **dangerous objects away** from them.
- Do **not restrain** their movements.
- Place them on their **side** to keep airway clear.
- **Time the seizure**; most last 1–3 minutes.
- **After seizure**: stay with them, speak calmly, and seek medical help if necessary.

Q2. Three out of four pain behaviors

Pain behaviors are **observable actions indicating pain**. Three main behaviors:

1. **Verbal complaints** – complaining, groaning, or moaning.
2. **Facial expressions** – grimacing or frowning.
3. **Protective behaviors** – limping, avoiding movement, or guarding the painful area.
(Fourth: Activity reduction, like decreased participation in normal tasks.)

Q3. Three types of health behaviors

Health behaviors are actions that **promote or maintain health**. Three types:

1. **Preventive behaviors** – e.g., vaccination, regular check-ups, handwashing.
2. **Illness behaviors** – recognizing symptoms and seeking medical care.
3. **Sick-role behaviors** – following treatment, resting, and adhering to doctor's advice.

Q4. Risk factors of cancer

Cancer risk factors include:

- **Lifestyle factors** – smoking, alcohol, unhealthy diet.
- **Environmental factors** – radiation, pollution, chemical exposure.
- **Genetic factors** – family history or inherited mutations.
- **Psychosocial factors** – stress, poor coping, lack of social support.

Q5. Note on modeling behavior in learning

- Modeling is **learning by observing others**.
- Individuals imitate behaviors demonstrated by **role models**.
- In health psychology, modeling influences **adoption of healthy habits** (e.g., exercising, diet, hand hygiene).
- Effective models are **similar, competent, and rewarded** for behavior.
- Helps in **behavioral interventions** and patient education.

Q6. Role of reinforcement in health behavior

- Reinforcement strengthens desired behavior.
- **Positive reinforcement:** Rewarding healthy behavior (praise, incentives).
- **Negative reinforcement:** Removing an unpleasant stimulus when healthy behavior occurs (e.g., reduced symptoms after medication adherence).
- Reinforcement helps **habit formation** and **maintenance of health behaviors**.
- Used in **behavioral therapy and health interventions**.

Q7. Types of problems in adjustment (in chronic illness)

Chronic illness can create **adjustment problems**:

1. **Emotional problems:** anxiety, depression, fear of death.
 2. **Social problems:** isolation, family conflicts, role changes.
 3. **Behavioral problems:** non-adherence to treatment, poor self-care.
 4. **Cognitive problems:** difficulty concentrating, negative thoughts about illness.
- Interventions include **counseling, support groups, coping strategies**.

Q8. Two types of behavioral coping in stress

Behavioral coping strategies help **manage stress through actions**. Two main types:

1. **Problem-focused coping** – Taking direct action to solve the stressor (e.g., creating a study schedule, seeking medical help).
 2. **Emotion-focused coping** – Actions to manage emotional response (e.g., exercising, relaxation techniques, social support).
- Purpose:* Both reduce stress effects and improve adaptation.

Q9. Effects of weight control

Maintaining healthy weight has **physical, psychological, and social benefits**:

- **Physical:** Reduces risk of diabetes, hypertension, heart disease, and joint problems.
- **Psychological:** Improves self-esteem, body image, and reduces depression/anxiety.
- **Social:** Better social interactions and confidence.
- **Overall:** Supports long-term health and improves quality of life.

Q10. Benign tumors & Malignant tumors

Benign tumors:

- Non-cancerous and **do not spread** to other parts.
- Grow slowly and have well-defined borders.
- Usually **not life-threatening**, but may cause problems if they press on organs.

Malignant tumors:

- Cancerous and **can invade nearby tissues** and **metastasize**.
- Grow rapidly and have irregular borders.
- Can be **life-threatening** and require aggressive treatment (surgery, chemotherapy, radiation).

Q11. How to do time management

Effective time management reduces stress and improves productivity:

1. **Set priorities** – focus on important and urgent tasks first.
 2. **Make a schedule** – allocate specific time slots for tasks.
 3. **Set realistic goals** – break tasks into smaller, manageable steps.
 4. **Avoid procrastination** – stay disciplined and minimize distractions.
 5. **Review and adjust** – check progress and revise plans if needed.
- Result:* Better control over time, less stress, improved performance.

Q12. Three cognitive techniques to measure pain

1. **Self-report questionnaires** – Patients rate their pain intensity or quality using standardized forms.
2. **Pain diaries** – Individuals record daily pain experiences, triggers, and severity.
3. **Verbal rating scales** – Using descriptive words (mild, moderate, severe) to express pain.

Purpose: Cognitive techniques capture subjective perception of pain and help guide treatment.

Q13. Three levels of prevention

1. **Primary prevention** – Prevents disease before it occurs (e.g., vaccination, healthy diet, exercise).
2. **Secondary prevention** – Early detection and treatment of disease (e.g., screening tests, blood pressure monitoring).
3. **Tertiary prevention** – Reduces complications and improves quality of life after disease onset (e.g., rehabilitation, therapy).

Q14. Transmission routes of HIV

HIV spreads through:

1. **Sexual contact** – Unprotected sex with an infected partner.
 2. **Blood** – Sharing needles, transfusions with infected blood.
 3. **Mother-to-child** – During pregnancy, childbirth, or breastfeeding.
- Prevention:* Safe sex, sterile needles, and mother treatment programs.

Q15. Three elements of time management

1. **Goal setting** – Define clear and achievable objectives.
2. **Prioritization** – Focus on important and urgent tasks first.
3. **Scheduling** – Allocate specific time blocks for tasks and review regularly.

Q16. Six risks of obesity and overweight

1. Cardiovascular diseases – heart attack, hypertension.
2. Type 2 diabetes.
3. Certain cancers – breast, colon.
4. Joint problems – osteoarthritis.
5. Sleep apnea and breathing difficulties.
6. Psychological issues – low self-esteem, depression.

Q17. Causes of asthma (Long question)

Asthma is caused by a combination of **genetic, environmental, and immunological factors**:

- **Genetic predisposition:** Family history increases risk.
- **Allergens:** Dust, pollen, mold, pet dander trigger attacks.
- **Air pollution:** Smoke, chemicals, and pollutants irritate airways.
- **Respiratory infections:** Viral infections can worsen asthma.
- **Exercise-induced asthma:** Physical activity may trigger symptoms.

Management: Avoid triggers, use inhalers, monitor symptoms, and follow medical guidance.

Q18. Pain rating scale (Long question)

Pain rating scales help **assess intensity and quality of pain**. Common scales:

1. **Visual Analog Scale (VAS):** Patient marks pain on a 10 cm line from “no pain” to “worst pain.”
2. **Numeric Rating Scale (NRS):** Rate pain from 0 (no pain) to 10 (worst pain).
3. **Faces Pain Scale (FPS):** Particularly for children; choose a face that matches pain.
Purpose: Guides treatment decisions and monitors progress.

Q19. Psychosocial aspects of stroke (Long question)

Stroke affects mental, emotional, and social functioning:

- **Emotional:** Anxiety, depression, frustration due to sudden disability.
- **Cognitive:** Memory loss, attention deficits, difficulty planning tasks.
- **Social:** Isolation, dependency on caregivers, changes in family dynamics.
- **Behavioral:** Difficulty in daily activities, decreased participation in social roles.
Interventions: Psychological counseling, rehabilitation, social support, and coping strategies improve adjustment.

Q20. Four types of pain

1. **Acute pain** – Short-term, caused by injury or surgery; disappears after healing.
2. **Chronic pain** – Lasts more than 3–6 months; persists despite treatment.
3. **Neuropathic pain** – Caused by nerve damage (e.g., diabetes, shingles).
4. **Visceral pain** – Originates from internal organs (e.g., heart attack, kidney stones).

Q21. Regimens for asthma

Asthma management includes:

1. **Preventive medications** – Inhaled corticosteroids to reduce inflammation.
2. **Reliever medications** – Short-acting bronchodilators for acute attacks.
3. **Allergen avoidance** – Dust, smoke, pets, pollen.
4. **Lifestyle modifications** – Regular exercise, balanced diet, stress management.
5. **Monitoring** – Keep track of symptoms and peak flow measurements.

Q22. Three symptoms of heart disease

1. **Chest pain or angina** – Pressure or tightness in the chest.
2. **Shortness of breath** – Difficulty in breathing during activity or rest.
3. **Fatigue** – Extreme tiredness or lack of energy.

Other possible symptoms: palpitations, dizziness, swelling in legs.

Q23. Role of stress on psychophysiological disorders

- Chronic stress activates **sympathetic nervous system** → increases heart rate, blood pressure.
- Leads to disorders like **hypertension, heart disease, ulcers, migraines**.
- Stress hormones (cortisol, adrenaline) disrupt **immune and endocrine function**.
- Stress can worsen **existing medical conditions** and reduce recovery.
- Managing stress through **relaxation, coping strategies, and therapy** helps prevent disease progression.

Q24. Interventions for stroke

- **Medical management:** Medication to prevent clot formation or reduce damage.
- **Physical rehabilitation:** Physiotherapy to regain motor function.
- **Occupational therapy:** Relearn daily activities and self-care skills.
- **Speech therapy:** Recover language and communication skills.
- **Psychological support:** Counseling, stress management, and family support.
- **Social support:** Peer groups and caregiver involvement.

Q25. Differentiate between HIV and AIDS (*Long question*)

Feature	HIV (Human Immunodeficiency Virus)	AIDS (Acquired Immunodeficiency Syndrome)
Definition	Virus that attacks immune system	Advanced stage of HIV infection
Immune impact	Reduces CD4+ T cells gradually	Severe depletion of CD4+ T cells
Symptoms	Often asymptomatic; mild flu-like illness	Opportunistic infections, severe illness
Diagnosis	Detected by HIV antibody or RNA test	Diagnosed when CD4 count <200 or AIDS-defining illness occurs
Treatment	Antiretroviral therapy (ART)	ART + treatment of opportunistic infections
Progression	Can remain latent for years	Life-threatening if untreated

Q26. Children copy bad behavior first or good?

- Children **learn by observation**, imitating parents, peers, teachers, or media models.
- They tend to **imitate what they see frequently**, whether good or bad.
- **Bad behavior** may appear first because it often draws **attention or immediate reinforcement**.
- With proper guidance, **positive reinforcement, modeling, and discipline**, children can learn **good behavior** effectively.
- *Conclusion:* Children copy both, but environment and reinforcement shape whether good or bad behavior dominates.

Q27. Five disorders that require medical care

1. **Diabetes Mellitus** – Requires medication, insulin, and monitoring.
2. **Hypertension** – Needs regular treatment to prevent complications.
3. **Asthma** – Managed through inhalers, medications, and lifestyle control.
4. **Heart disease** – Requires medication, surgery, or lifestyle interventions.
5. **Cancer** – Requires chemotherapy, radiotherapy, or surgery and ongoing medical supervision.

Q28. Why social support is important in chronic illness

- Provides **emotional support**, reducing stress, anxiety, and depression.
- Encourages **adherence to treatment** and lifestyle modifications.
- Offers **practical help** like transport, meals, or reminders for medications.
- Enhances **self-esteem** and sense of belonging.
- Improves **recovery outcomes and quality of life** in patients with chronic conditions.

Q29. Risk factors of cancer

- **Lifestyle:** Smoking, alcohol, unhealthy diet, lack of exercise.

- **Genetic:** Family history, inherited mutations.
- **Environmental:** Radiation, pollution, chemicals.
- **Infections:** HPV, Hepatitis B & C.
- **Psychosocial:** Chronic stress, poor coping, low social support.

Q30. Three types of pain behavior

1. **Verbal complaints** – moaning, complaining of pain.
2. **Facial expressions** – grimacing, frowning, or wincing.
3. **Protective behaviors** – limping, guarding, avoiding activity.

Purpose: Indicates pain intensity, helps in assessment and treatment.

Q31. Types of adjustment in chronic illness

1. **Emotional adjustment** – coping with anxiety, depression, and fear.
2. **Behavioral adjustment** – adherence to treatment, self-care routines.
3. **Social adjustment** – maintaining relationships, social participation.
4. **Cognitive adjustment** – understanding illness, making informed decisions.

Q32. Coping with illness behavior

- **Problem-focused coping:** Active steps to manage illness (e.g., taking medication, seeking help).
- **Emotion-focused coping:** Managing emotions related to illness (e.g., relaxation, counseling).
- **Avoidance coping:** Ignoring or denying illness (less effective).
Goal: Reduce stress, maintain function, and improve quality of life.

Q33. How school contributes to health/chronic problems

- **Promotes healthy behaviors:** Nutrition, exercise, hygiene education.
- **Early detection:** Screening for vision, hearing, and developmental issues.
- **Stress management:** Teaching coping skills, emotional support.
- **Peer influence:** Encourages socialization and positive habits.
- **Prevention programs:** Vaccination drives, anti-smoking campaigns, mental health awareness.

Q34. Types of coping

Coping strategies help manage stress and illness. Major types:

1. **Problem-focused coping** – Taking action to solve the problem causing stress (e.g., following a treatment plan).
2. **Emotion-focused coping** – Managing emotional responses to stress (e.g., relaxation, talking to friends).

3. **Avoidance coping** – Ignoring or denying the problem (less effective).
4. **Behavioral coping** – Actions like exercising, seeking medical help, or using protective behaviors.
5. **Cognitive coping** – Changing thoughts about the problem to reduce stress.

Purpose: Improves adaptation, mental health, and recovery in chronic illness.

Q35. How can we assess people's pain

- **Self-report measures** – Patients describe intensity, quality, and location of pain (e.g., questionnaires).
- **Visual Analog Scale (VAS)** – Mark a line representing pain from “no pain” to “worst pain.”
- **Numeric Rating Scale (NRS)** – Rate pain from 0–10.
- **Faces Pain Scale** – Often used for children; choose the face that matches pain.
- **Behavioral observation** – Grimacing, guarding, limping, or activity reduction.

Purpose: Helps clinicians monitor and treat pain effectively.

Q36. Adjusting to chronic illness

Adjustment involves **adapting physically, emotionally, socially, and cognitively:**

1. **Emotional adjustment** – Coping with anxiety, depression, frustration, and fear.
2. **Behavioral adjustment** – Following treatment, self-care, lifestyle modifications.
3. **Social adjustment** – Maintaining relationships, seeking support, participating in social life.
4. **Cognitive adjustment** – Understanding illness, planning ahead, problem-solving.

Goal: Improve quality of life, reduce stress, and maintain function.

Q37. Diabetes-related question

- **Type 1 Diabetes:** Insulin-dependent, usually diagnosed in children/young adults.
- **Type 2 Diabetes:** Often adult-onset, related to obesity and lifestyle.

Management:

1. **Medication:** Insulin or oral hypoglycemics.
2. **Diet:** Balanced diet, controlled sugar intake.
3. **Exercise:** Regular physical activity to regulate blood sugar.
4. **Monitoring:** Blood glucose checks.
5. **Education:** Awareness about symptoms, complications, and self-care.

Goal: Prevent complications and maintain health.

Q38. Types of adjustment problems in chronic illness

1. **Emotional problems:** Anxiety, depression, anger, or frustration.
2. **Behavioral problems:** Non-adherence to treatment, poor self-care.
3. **Social problems:** Isolation, dependency, family conflict.
4. **Cognitive problems:** Difficulty understanding illness, negative thoughts, poor decision-making.

Intervention: Counseling, social support, patient education, and coping strategies.

Q39. Rehabilitation of stroke

- **Physical therapy:** Regain mobility, strength, balance.
- **Occupational therapy:** Relearn daily activities and independence.
- **Speech therapy:** Recover communication and swallowing skills.
- **Psychological support:** Counseling, stress management, coping strategies.
- **Family and social support:** Encourages motivation, participation, and adjustment.

Q40. How schools promote health

- **Health education:** Nutrition, hygiene, sexual health, mental health awareness.
- **Physical activity programs:** Exercise, sports, and active lifestyle.
- **Screening programs:** Vision, hearing, and vaccination campaigns.
- **Stress management:** Teaching coping skills and emotional regulation.
- **Promote social well-being:** Encourage peer support, teamwork, and healthy habits.

Q41. How can you control your weight

- **Balanced diet:** Eat nutritious, low-fat, high-fiber meals.
- **Regular exercise:** Cardiovascular, strength, and flexibility training.
- **Monitor intake:** Keep track of calories and food habits.
- **Behavioral strategies:** Avoid emotional eating, plan meals, set realistic goals.
- **Adequate sleep and stress management:** Poor sleep and high stress can increase weight gain.

Q42. The study of death or dying is called

- **Thanatology** – The scientific study of death, dying, and bereavement.
- Focuses on **psychological, social, and cultural aspects** of end-of-life, grief, and coping.
- Helps **health professionals** manage patients, families, and communities facing terminal illness.

Q43. Components of food (Nutrition & Health)

1. **Carbohydrates** – Main energy source; found in grains, fruits, and vegetables.
2. **Proteins** – Build and repair body tissues; found in meat, legumes, dairy.
3. **Fats** – Energy storage, insulation, and hormone production; found in oils, nuts, dairy.
4. **Vitamins** – Organic compounds essential for metabolism and immunity.

5. **Minerals** – Inorganic elements for bones, teeth, nerve function (e.g., calcium, iron).
6. **Water** – Regulates body temperature, transports nutrients, removes waste.
7. **Fiber** – Indigestible plant material; aids digestion, prevents constipation, regulates blood sugar.

MCQs are often framed around these components and their functions.

Q44. Why social support is essential for illness behavior

- Provides **emotional support**, reducing stress, anxiety, and depression.
- Encourages **treatment adherence** and positive health behaviors.
- Offers **practical help** (transport, reminders, daily care).
- Enhances **self-esteem** and coping ability.
- Improves **recovery outcomes and quality of life** for chronically ill patients.

Q45. Can lifestyle affect health?

- Yes, lifestyle choices **directly impact physical and mental health**.
- **Diet:** Balanced diet prevents obesity, diabetes, cardiovascular diseases.
- **Exercise:** Regular activity improves cardiovascular health, mood, and immunity.
- **Substance use:** Smoking and alcohol increase risk of cancer, liver, and lung diseases.
- **Sleep:** Adequate sleep improves cognitive function, mood, and metabolism.
- **Stress management:** Reduces risk of psychophysiological disorders.

Q46. Name three chronic illnesses

1. **Diabetes mellitus** – Type 1 or Type 2; affects blood sugar regulation.
2. **Hypertension** – High blood pressure; risk factor for heart disease and stroke.
3. **Chronic obstructive pulmonary disease (COPD)** – Long-term lung disease causing breathing difficulties.

Q47. Four points about diabetes

1. **Definition:** Chronic condition with impaired insulin production or utilization.
2. **Types:** Type 1 (insulin-dependent) and Type 2 (often lifestyle-related).
3. **Symptoms:** Frequent urination, excessive thirst, fatigue, blurred vision.
4. **Management:** Medication (insulin/oral drugs), diet control, regular exercise, and blood sugar monitoring.

Q48. Four interventions for stroke

1. **Medical treatment:** Medication to reduce clotting or prevent further damage.
2. **Physical therapy:** Improve strength, mobility, and coordination.
3. **Occupational therapy:** Relearn daily tasks and self-care skills.
4. **Speech therapy:** Recover communication and swallowing abilities.
(Additional: Psychological counseling and family support enhance recovery.)

Q49. Six causes of asthma

1. **Genetic predisposition:** Family history increases risk.
2. **Allergens:** Dust, pollen, pet dander, mold.
3. **Air pollution:** Smoke, chemicals, environmental toxins.
4. **Respiratory infections:** Viral infections triggering attacks.
5. **Exercise-induced triggers:** Physical activity can provoke symptoms.
6. **Stress and emotional factors:** Can exacerbate asthma symptoms.

Q50. Herbal products (senior-based question)

- Many seniors use **herbal supplements** for wellness or chronic conditions.
- Examples: **Ginkgo biloba** (memory), **Turmeric/Curcumin** (anti-inflammatory), **Garlic** (heart health).
- Benefits are sometimes **mild and supportive**, not replacements for prescribed medications.
- **Caution:** Herbal products may **interact with prescribed medications**; consult healthcare providers.

Q51. All about herbal products

- Herbal products are **plant-based remedies** used for prevention or treatment.
- Common forms: teas, capsules, powders, extracts.
- Purported benefits: immune support, inflammation reduction, mental health enhancement.
- Safety concerns: dosing, contamination, and interactions with drugs.
- Recommended to use **evidence-based herbal products** under medical supervision.

Q52. Three pain behaviors

1. **Verbal complaints:** Saying “it hurts,” groaning, moaning.
2. **Facial expressions:** Grimacing, frowning, wincing.
3. **Protective behaviors:** Guarding the affected area, limping, avoiding movement.

Q53. Three levels of prevention

1. **Primary prevention:** Prevent disease before it occurs (e.g., vaccination, healthy diet, exercise).
2. **Secondary prevention:** Early detection (screening, routine check-ups).
3. **Tertiary prevention:** Reduce complications and improve quality of life after disease onset (rehabilitation, therapy).

Q54. Three benefits of exercise for weight loss

1. **Burns calories:** Helps reduce fat and maintain healthy weight.
2. **Improves metabolism:** Increases basal metabolic rate and energy use.

3. **Enhances muscle tone:** Preserves lean body mass and improves body composition.

Q55. Three pain behaviors

(Repeated but exam often asks again, write concisely)

- Verbal complaints
- Facial expressions
- Protective behaviors

Q56. Role of reinforcement in coping with health and illness

- **Positive reinforcement:** Encourages healthy behaviors by providing rewards (e.g., praise for taking medication regularly).
- **Negative reinforcement:** Reduces unhealthy behavior by removing unpleasant outcomes (e.g., reducing pain after exercising or following diet).
- **Purpose:** Reinforcement strengthens adherence to treatment, promotes recovery, and helps maintain lifestyle changes in chronic illness.

Q57. Fats and their types

- **Definition:** Fats are a macronutrient providing energy, insulation, and aiding hormone production.
- **Types:**
 1. **Saturated fats:** Solid at room temperature; found in butter, cheese, meat.
 2. **Unsaturated fats:** Liquid at room temperature; includes:
 - **Monounsaturated:** Olive oil, nuts.
 - **Polyunsaturated:** Fish oil, sunflower oil.
 3. **Trans fats:** Artificial fats, found in processed foods; harmful.

Q58. Psychosocial aspects of stroke

- **Emotional:** Anxiety, depression, frustration due to sudden disability.
- **Cognitive:** Memory loss, attention deficits, difficulty planning tasks.
- **Social:** Isolation, dependency on caregivers, family dynamic changes.
- **Behavioral:** Difficulty in daily activities, decreased social participation.
- **Interventions:** Counseling, social support, rehabilitation programs, coping strategies.

Q59. Modeling behavior in learning

- **Definition:** Learning occurs by observing and imitating others' behavior.
- **Process:** Attention → Retention → Reproduction → Motivation.
- **Example in health:** Children copy parents brushing teeth, washing hands, or exercising.
- **Impact:** Positive models encourage healthy habits; negative models may promote risky or unhealthy behavior.

Q60. Pain rating / measuring scales

- **Visual Analog Scale (VAS):** Mark pain on a 10 cm line from “no pain” to “worst pain.”
- **Numeric Rating Scale (NRS):** Rate pain from 0–10.
- **Faces Pain Scale (FPS):** Choose face matching pain intensity (for children).
- **McGill Pain Questionnaire:** Measures sensory, affective, and evaluative aspects of pain.
- **Behavioral observation:** Watching protective behaviors, grimacing, or movement limitation.

Q61. How chronic illness is treated in social environments

- **Social support:** Family, friends, and community aid recovery and adherence to treatment.
- **Education:** Informing patients and caregivers about disease management.
- **Behavioral strategies:** Encourage routines, medication adherence, diet, and exercise.
- **Rehabilitation programs:** Group therapy, peer support, and occupational therapy.
- **Psychological support:** Counseling, stress management, and coping skills training.

Q62. Epilepsy and two types

- **Definition:** Epilepsy is a neurological disorder characterized by recurrent seizures.
- **Types:**
 1. **Generalized seizures:** Affect the whole brain (e.g., tonic-clonic seizures).
 2. **Focal/partial seizures:** Affect a specific area of the brain (e.g., simple or complex partial seizures).
- **Management:** Medication (antiepileptics), lifestyle adjustments, and sometimes surgery.

Q63. Biological aspects of disease

- Biological factors influence susceptibility, progression, and recovery. Examples:
 - **Genetic predisposition** – Family history of diabetes, hypertension.
 - **Pathophysiology** – How organs and systems are affected (e.g., stroke affects brain neurons).
 - **Immune function** – Weakened immunity increases infection risk.
 - **Neurochemical factors** – Stress hormones (cortisol, adrenaline) affect heart and metabolism.

Q64. Types of coping

1. **Problem-focused coping** – Active efforts to solve the problem causing stress (e.g., following a treatment plan).
2. **Emotion-focused coping** – Managing emotional response to stress (e.g., relaxation, meditation, counseling).
3. **Avoidance coping** – Ignoring or denying the problem (less effective).

4. **Behavioral coping** – Actions like exercising, seeking medical help, or using protective behaviors.
5. **Cognitive coping** – Changing thoughts about the stressor to reduce impact.

Q65. Six risk factors of cancer

1. **Lifestyle:** Smoking, alcohol, poor diet, physical inactivity.
2. **Genetic factors:** Family history, inherited mutations.
3. **Environmental exposures:** Radiation, chemicals, pollution.
4. **Infections:** HPV, Hepatitis B & C.
5. **Age:** Risk increases with age.
6. **Psychosocial factors:** Chronic stress, poor coping, low social support.

Q66. Name 3 types of lipoproteins

1. **Low-density lipoprotein (LDL):** “Bad cholesterol,” deposits fat in arteries.
2. **High-density lipoprotein (HDL):** “Good cholesterol,” removes fat from arteries.
3. **Very-low-density lipoprotein (VLDL):** Transports triglycerides from liver to tissues.

Q67. Six adjustment problems in chronic illness

1. **Emotional:** Anxiety, depression, frustration.
2. **Behavioral:** Non-adherence to treatment, poor self-care.
3. **Cognitive:** Difficulty understanding illness, poor decision-making.
4. **Social:** Isolation, dependency, family conflict.
5. **Physical:** Fatigue, reduced mobility, pain management issues.
6. **Occupational/financial:** Reduced work capacity, income loss, stress about expenses.

Q68. Scenario-based behavior question

- Often requires applying **health psychology concepts:** e.g., a patient refusing medication.
- **Steps to approach:**
 1. Identify **problem behavior** (non-adherence, unhealthy habit).
 2. Determine **psychological cause** (stress, fear, lack of knowledge).
 3. Suggest **intervention:** education, reinforcement, counseling, social support.
 4. Evaluate outcome through observation and feedback.

Q69. Importance of exercise in modern lifestyle (*Long question*)

- **Physical benefits:** Maintains healthy weight, strengthens muscles and bones, improves cardiovascular health.
- **Mental benefits:** Reduces anxiety, depression, stress; improves mood and cognitive function.
- **Chronic disease prevention:** Lowers risk of diabetes, hypertension, obesity, and heart disease.
- **Enhances immune function:** Improves overall resistance to infections.

- **Lifestyle integration:** Helps counter sedentary behavior common in modern work and school life.

Q70. Does lifestyle affect health? (*Long question*)

- **Yes, lifestyle is a major determinant of health.**
- **Diet:** Balanced diet prevents obesity, diabetes, and heart disease.
- **Exercise:** Regular physical activity maintains fitness and mental well-being.
- **Substance use:** Smoking, alcohol, and drugs increase disease risk.
- **Sleep & stress:** Adequate sleep and stress management improve immunity and cognitive function.
- **Social and behavioral factors:** Social support and healthy habits promote long-term wellness.

Q71. Why social support is essential in illness

- Provides **emotional support** – reduces anxiety, depression, and stress.
- Encourages **adherence to treatment** – patients follow prescriptions and therapy.
- Offers **practical help** – transport, meal preparation, reminders.
- Enhances **self-esteem** and coping ability.
- Improves **recovery outcomes, quality of life, and adjustment** to chronic illness.

Q72. Causes of asthma

1. **Genetic predisposition:** Family history increases risk.
2. **Allergens:** Dust, pollen, pet dander, mold.
3. **Air pollution:** Smoke, chemicals, environmental toxins.
4. **Respiratory infections:** Viral or bacterial infections triggering attacks.
5. **Exercise-induced triggers:** Physical activity can provoke symptoms.
6. **Stress and emotional factors:** Psychological stress can worsen asthma.

Q73. Definition of fats and their types

- **Definition:** Fats are macronutrients that provide energy, insulation, and aid in hormone production.
- **Types:**
 1. **Saturated fats:** Solid at room temperature; found in butter, cheese, meat.
 2. **Unsaturated fats:** Liquid at room temperature; includes:
 - Monounsaturated: Olive oil, nuts
 - Polyunsaturated: Fish oil, sunflower oil
 3. **Trans fats:** Artificial fats in processed foods; harmful to health.

Q74. Psychosocial aspects of stroke

- **Emotional:** Anxiety, depression, frustration, grief over lost abilities.
- **Cognitive:** Memory loss, attention deficits, difficulty in planning.

- **Social:** Isolation, dependency, changes in family dynamics.
- **Behavioral:** Difficulty in daily activities, reduced social participation.
- **Interventions:** Counseling, social support, rehabilitation, coping strategies.

Q75. Pain behaviors are reinforced (short note)

- Pain behaviors (grimacing, complaining, guarding) can be **reinforced positively or negatively**.
- **Positive reinforcement:** Attention or care increases display of pain behavior.
- **Negative reinforcement:** Avoiding tasks or obligations strengthens pain expression.
- **Goal:** Understanding reinforcement helps manage chronic pain effectively.

Q76. Asthma regimens

1. **Preventive medications:** Inhaled corticosteroids to reduce airway inflammation.
2. **Reliever medications:** Short-acting bronchodilators for acute attacks.
3. **Allergen avoidance:** Dust, smoke, pollen, pets.
4. **Lifestyle modifications:** Regular exercise, balanced diet, stress management.
5. **Monitoring:** Track symptoms and peak flow readings to adjust treatment.

Q77. Behavior identification (conceptual scenario)

- Identify the **target behavior** in a scenario (e.g., patient refusing medication, skipping exercise, overeating).
- Analyze **causal factors:** psychological, social, environmental, cognitive.
- Suggest **intervention:** reinforcement, education, counseling, or social support.
- Evaluate outcome using **feedback and observation**.

Q78. Symptoms of heart disease

1. **Chest pain or angina:** Pressure or tightness in the chest.
2. **Shortness of breath:** Difficulty in breathing during activity or rest.
3. **Fatigue:** Extreme tiredness or lack of energy.
4. **Other symptoms:** Palpitations, dizziness, swelling in legs.

Q79. Myth: Natural or herbal weight loss products are safe and effective

- **Reality:** Many herbal products are **not scientifically validated** for weight loss.
- Some may interact with medications, causing **side effects or toxicity**.
- Effectiveness is often **mild or temporary**, not a replacement for diet and exercise.
- **Recommendation:** Use only **evidence-based supplements** under medical supervision.
- Encourage **lifestyle changes** (diet, exercise, sleep, stress management) for safe, effective weight control.

Q80. Two symptoms of heart disease

1. **Chest pain or angina:** Pressure, squeezing, or tightness in the chest.
2. **Shortness of breath:** Difficulty breathing during activity or even at rest.

(Additional symptoms for knowledge: fatigue, palpitations, dizziness, swelling in legs.)

Q81. Three elements of time management

1. **Planning:** Creating schedules, setting goals, and prioritizing tasks.
2. **Organization:** Arranging tasks and resources efficiently to avoid waste of time.
3. **Monitoring and adjustment:** Tracking progress, avoiding distractions, and making timely changes to plans.

Q82. Suggestions or steps to help an epileptic person

1. **Stay calm** and keep the person safe from injury (move objects away).
2. **Do not restrain** their movements during a seizure.
3. **Time the seizure** and monitor duration.
4. **Place them on their side** to maintain open airway.
5. **Do not put anything in the mouth** during a seizure.
6. **After seizure:** Offer reassurance, allow rest, and seek medical help if needed.
7. **Education:** Family and friends should learn seizure first aid and emergency procedures.

Q83. How patients and families cope with terminal illness

- **Emotional coping:** Counseling, expressing feelings, acceptance, stress management.
- **Social support:** Family, friends, and community provide practical and emotional help.
- **Cognitive coping:** Understanding prognosis, planning for end-of-life care.
- **Behavioral coping:** Maintaining routines, adhering to treatment, symptom management.
- **Spiritual coping:** Religious or spiritual beliefs provide comfort and meaning.
- **Interventions:** Hospice care, palliative care, support groups improve quality of life.

Q84. Techniques to assess people's pain

1. **Self-report measures:** Questionnaires, pain diaries.
2. **Visual Analog Scale (VAS):** Marking pain on a line from “no pain” to “worst pain.”
3. **Numeric Rating Scale (NRS):** Rating pain from 0–10.
4. **Faces Pain Scale (FPS):** For children; choose face that matches pain intensity.
5. **Behavioral observation:** Grimacing, guarding, reduced activity, limping.

Q85. How schools contribute to promoting health

- **Health education:** Nutrition, hygiene, mental health awareness, sexual health.
- **Physical activity programs:** Exercise, sports, and active lifestyles.
- **Screening programs:** Vision, hearing, vaccination campaigns.
- **Stress management:** Teach coping skills, emotional regulation.
- **Social well-being:** Encourage peer support, teamwork, and healthy habits.

- **Healthy environment:** Safe classrooms, clean water, and sanitary facilities.

Q86. Natural and herbal products are safe and effective – Facts against this myth

1. Many **herbal products lack scientific validation** for safety and efficacy.
2. Some may **interact with prescribed medications**, causing side effects or toxicity.
3. Effectiveness is often **mild or temporary**, not a replacement for medical treatment.
4. Quality control is **not always regulated**, leading to contamination or incorrect dosing.
5. Safe and effective weight loss or health improvement **requires evidence-based lifestyle changes**: diet, exercise, and stress management.

Q87. AIDS transmission: 3 ways

1. **Sexual contact:** Unprotected sex with an infected partner.
2. **Blood contact:** Sharing contaminated needles, blood transfusions, or organ transplants.
3. **Mother-to-child transmission:** During pregnancy, childbirth, or breastfeeding.

Q88. Two types of coping

1. **Problem-focused coping:** Actively addressing the stressor (e.g., following medical advice, lifestyle changes).
2. **Emotion-focused coping:** Managing emotional responses (e.g., meditation, counseling, relaxation techniques).

Q89. What is epilepsy?

- **Definition:** A neurological disorder characterized by **recurrent, unprovoked seizures** caused by abnormal electrical activity in the brain.
- **Symptoms:** Convulsions, loss of consciousness, staring spells, or unusual sensations.
- **Management:** Medication, lifestyle adjustments, and sometimes surgery.

Q90. Two types of epilepsy

1. **Generalized seizures:** Affect the entire brain (e.g., tonic-clonic seizures, absence seizures).
2. **Focal/partial seizures:** Affect specific brain regions (e.g., simple partial, complex partial seizures).

Q91. Benefits of exercise in modern lifestyle (*Long question*)

- **Physical health:** Maintains healthy weight, strengthens muscles and bones, improves cardiovascular fitness.
- **Mental health:** Reduces anxiety, depression, and stress; improves mood and cognitive function.
- **Chronic disease prevention:** Lowers risk of diabetes, hypertension, obesity, and heart disease.

- **Immune support:** Enhances resistance to infections.
- **Lifestyle adaptation:** Counters sedentary behavior from modern work and school routines.
- **Social benefits:** Participation in sports improves teamwork and social interaction.

Q92. Benign and malignant tumors (*Long question*)

- **Benign tumors:**
 - Non-cancerous growths.
 - Grow slowly, do not invade nearby tissues.
 - Usually not life-threatening, removable by surgery.
- **Malignant tumors:**
 - Cancerous growths.
 - Rapid growth, invade surrounding tissues, and may metastasize (spread).
 - Life-threatening, require surgery, chemotherapy, or radiation.

Q93. Measures for a patient suffering from an epilepsy attack (*Long question*)

1. **Ensure safety:** Remove harmful objects from the area.
2. **Do not restrain movements** during seizure.
3. **Place the person on their side** to maintain airway.
4. **Monitor duration** of seizure and note symptoms.
5. **Do not put anything in the mouth.**
6. **After seizure:** Offer reassurance, allow rest, and seek medical help if needed.
7. **Education:** Family and caregivers should learn first aid for seizures.

Q94. Pain measuring scales

1. **Visual Analog Scale (VAS):** Patient marks pain on a line from “no pain” to “worst pain.”
2. **Numeric Rating Scale (NRS):** Rate pain from 0 (no pain) to 10 (worst pain).
3. **Faces Pain Scale (FPS):** Especially for children; choose a face matching pain intensity.
4. **McGill Pain Questionnaire:** Assesses sensory, affective, and evaluative components of pain.
5. **Behavioral observation:** Observing protective behaviors, grimacing, limping, or reduced activity.

Q95. Enlist any 3 reactions to chronic illness

1. **Emotional reaction:** Anxiety, depression, fear, or frustration.
2. **Behavioral reaction:** Avoiding treatment, non-adherence to medication, or unhealthy coping.
3. **Cognitive reaction:** Difficulty accepting illness, negative thoughts, or denial.

Q96. Three leading causes of death in developing countries

1. **Cardiovascular diseases** – Heart attack, stroke.
2. **Infectious diseases** – Tuberculosis, HIV/AIDS, malaria.
3. **Respiratory diseases** – Chronic obstructive pulmonary disease, pneumonia.

Q97. Interventions for stroke

1. **Medical treatment:** Medications to reduce clotting, prevent further damage.
2. **Physical therapy:** Strength, mobility, and coordination training.
3. **Occupational therapy:** Relearning daily tasks and self-care skills.
4. **Speech therapy:** Recover communication and swallowing abilities.
5. **Psychological support:** Counseling to manage depression, anxiety, and stress.
6. **Social support:** Family involvement and rehabilitation groups.

Q98. How stress affects psychophysiology

- **Hormonal changes:** Elevated cortisol and adrenaline.
- **Cardiovascular system:** Increased heart rate, blood pressure, risk of heart disease.
- **Immune system:** Reduced immunity, more prone to infections.
- **Digestive system:** Ulcers, indigestion, appetite changes.
- **Musculoskeletal system:** Muscle tension, headaches, fatigue.
- **Cognitive function:** Poor concentration, memory difficulties, anxiety, depression.

Q99. How lifestyle affects health (detailed)

- **Diet:** Balanced diet prevents obesity, diabetes, and cardiovascular disease.
- **Exercise:** Regular physical activity maintains fitness, weight, and mental health.
- **Substance use:** Avoiding smoking, alcohol, and drugs reduces disease risk.
- **Sleep:** Adequate sleep improves immunity, mood, and cognitive function.
- **Stress management:** Coping techniques reduce risk of psychosomatic and chronic diseases.
- **Social behavior:** Strong social support promotes mental health and adherence to healthy behaviors.
- **Environmental factors:** Clean water, hygiene, and safe living conditions reduce disease burden.

Q100. Two types of problem-focused coping

1. **Active coping:** Taking steps to directly solve the stressor (e.g., adhering to treatment, planning meals, following exercise routine).
2. **Planning:** Strategically preparing for challenges and creating solutions to minimize impact of stressors.

Q101. Three tips to manage weight

1. **Balanced diet:** Include fruits, vegetables, whole grains, lean proteins; reduce sugar and saturated fats.

2. **Regular physical activity:** Combine aerobic exercises (walking, cycling) with strength training.
3. **Lifestyle habits:** Proper sleep, stress management, avoid late-night eating, and track progress.

Q102. Types of pain behavior

1. **Verbal complaints:** Complaining about pain, moaning, or verbal expression.
2. **Facial expressions:** Grimacing, frowning, wincing.
3. **Protective behaviors:** Guarding, limping, avoiding movement.
4. **Social withdrawal:** Avoiding social interactions or activities due to pain.
5. **Activity reduction:** Reducing physical activity, work, or exercise.

Q103. Exercise benefits for weight control

- **Energy expenditure:** Burns calories, creating a calorie deficit for weight loss.
- **Muscle building:** Increases metabolism and maintains lean body mass.
- **Appetite regulation:** Helps balance hunger hormones.
- **Prevention of weight regain:** Sustains long-term weight management.
- **Mental health benefit:** Reduces stress-induced overeating and improves mood.

Q104. Illness-related factors that influence coping process

- **Severity of illness:** More severe conditions require stronger coping strategies.
- **Duration of illness:** Chronic illnesses require long-term coping mechanisms.
- **Symptom visibility:** Obvious symptoms may affect social support and self-esteem.
- **Predictability and controllability:** Unpredictable illnesses increase stress.
- **Treatment complexity:** Complicated treatments can increase emotional and behavioral strain.

Q105. Stress-related conceptual question

- **Definition:** Stress is the psychological and physiological response to perceived threats or challenges.
- **Types:** Acute (short-term) vs chronic (long-term).
- **Effects on health:**
 - Emotional: Anxiety, depression.
 - Physiological: Increased heart rate, cortisol, blood pressure.
 - Behavioral: Poor sleep, unhealthy eating, substance use.
- **Coping strategies:** Problem-focused (addressing cause), emotion-focused (managing response), relaxation, social support.

Q106. Chronic diseases

- **Definition:** Long-term illnesses that often require ongoing management and care.
- **Examples:** Diabetes, hypertension, heart disease, asthma, arthritis.

- **Impact:** Physical limitation, emotional stress, financial burden, social dependency.
- **Management:** Medication adherence, lifestyle modifications, social and family support, rehabilitation programs.

Q107. Interventions for strokes

1. **Medical treatment:** Clot-dissolving medication, surgery if needed.
2. **Physical therapy:** Restore mobility, balance, and strength.
3. **Occupational therapy:** Help in daily living activities (e.g., dressing, cooking).
4. **Speech therapy:** Regain communication and swallowing ability.
5. **Psychological support:** Counseling for depression, anxiety, or frustration.
6. **Social support:** Family education and peer support groups to improve adherence and recovery.

Q108. Cardiac rehabilitation

- **Definition:** A structured program to help patients recover after a cardiac event (heart attack, surgery, or heart failure).
- **Goals:**
 1. Restore physical fitness and cardiovascular health.
 2. Reduce risk factors like high blood pressure, cholesterol, and obesity.
 3. Improve mental well-being and coping with heart disease.
 4. Promote long-term adherence to healthy lifestyle habits.
- **Components:**
 - **Medical management:** Monitoring vitals, medications, regular check-ups.
 - **Exercise training:** Gradual aerobic and strength training.
 - **Nutritional counseling:** Balanced diet, weight management.
 - **Psychological support:** Stress management, anxiety, depression, and social support.
 - **Education:** Awareness about heart disease, lifestyle modification, and warning signs.

Q109. Illness-related behavior

- **Definition:** How individuals perceive, respond to, and act regarding their health and illness.
- **Types of illness-related behavior:**
 1. **Seeking medical care:** Visiting doctors, hospitals, or therapists.
 2. **Adherence to treatment:** Following prescribed medications and lifestyle changes.
 3. **Symptom monitoring:** Observing bodily changes, pain, or warning signs.
 4. **Health-enhancing behaviors:** Exercise, diet, relaxation.
 5. **Health-impairing behaviors:** Smoking, alcohol, non-adherence, risky activities.
- **Factors influencing behavior:** Illness severity, social support, health beliefs, stress, coping skills.

Q110. Fats and its types

- **Definition:** Fats are essential macronutrients providing energy, insulation, and aiding hormone production.
- **Functions:** Energy storage, cell membrane integrity, absorption of fat-soluble vitamins (A, D, E, K).
- **Types:**
 1. **Saturated fats:** Found in meat, butter, cheese; solid at room temperature. High intake linked to heart disease.
 2. **Unsaturated fats:** Liquid at room temperature; includes:
 - **Monounsaturated fats:** Olive oil, nuts; reduce LDL cholesterol.
 - **Polyunsaturated fats:** Fish oil, sunflower oil; essential for brain and heart health.
 3. **Trans fats:** Found in processed foods; increase bad cholesterol (LDL) and risk of heart disease.

Q111. Pain measurements

- **Definition:** Quantifying pain intensity and experience to guide treatment.
- **Techniques:**
 1. **Self-report scales:** Questionnaires or diaries where patients describe pain.
 2. **Visual Analog Scale (VAS):** Line from “no pain” to “worst pain,” patient marks intensity.
 3. **Numeric Rating Scale (NRS):** Rate pain 0–10.
 4. **Faces Pain Scale (FPS):** Used for children; select a face representing pain intensity.
 5. **Behavioral observation:** Noting grimacing, guarding, limping, or reduced activity.
 6. **McGill Pain Questionnaire:** Measures sensory, affective, and evaluative components of pain.

Q112. Herbal treatment (Short & Long)

- **Short:** Use of plant-based products to prevent, manage, or treat illnesses.
- **Long:**
 - Herbal treatments include remedies from plants such as ginseng, turmeric, garlic, or aloe vera.
 - They are used for **weight management, immunity, digestion, and chronic illnesses**.
 - Effectiveness **varies**; scientific validation is often limited.
 - **Precautions:** Can interact with medications, may be contaminated or overdosed.
 - Recommended to use under **professional guidance** along with lifestyle modifications.

Q113. Amina sees an actress on TV doing makeup, she also does the same – learning type

- **Learning type: Observational learning (Modeling)**
 - According to **Bandura's Social Learning Theory**, people learn behaviors by **observing and imitating models**.
 - Key steps: Attention → Retention → Reproduction → Motivation.
 - Example: Amina observes actress (model), remembers steps, and imitates behavior.

Q114. Acute and chronic clinical pain

- **Acute pain:**
 - Short-term, sudden onset, usually linked to injury or surgery.
 - Protective, alerts body to danger.
 - Resolves with healing.
- **Chronic pain:**
 - Long-term (3+ months), persistent beyond normal healing.
 - May result from chronic illness (arthritis, cancer).
 - Leads to emotional distress, depression, reduced quality of life.

Q115. Pain coping strategies

1. **Problem-focused coping:** Addressing cause (medications, therapy, lifestyle changes).
2. **Emotion-focused coping:** Relaxation, meditation, distraction, seeking social support.
3. **Cognitive strategies:** Positive self-talk, reframing, mindfulness.
4. **Behavioral strategies:** Activity pacing, exercise, physical therapy.
5. **Social strategies:** Family support, group therapy, professional counseling.

Q116. Three disorders in developed countries causing death

1. **Cardiovascular diseases:** Heart attack, stroke.
2. **Cancer:** Lung, breast, colon, or other types.
3. **Diabetes-related complications:** Kidney failure, cardiovascular complications.

Q117. Social support in chronic illness

- **Definition:** Assistance from family, friends, or community in coping with illness.
- **Importance:**
 - Reduces stress and emotional burden.
 - Improves treatment adherence.
 - Enhances recovery and rehabilitation outcomes.
 - Provides practical help (transport, daily tasks) and emotional comfort.
- **Types:** Emotional, informational, instrumental, appraisal support.

Q118. Benign and malignant tumor difference

Feature	Benign Tumor	Malignant Tumor
Growth	Slow	Rapid

Feature	Benign Tumor	Malignant Tumor
Spread	Does not invade surrounding tissues	Invades surrounding tissues & may metastasize
Life Threat	Usually not life-threatening	Potentially life-threatening
Treatment	Often removable by surgery	Surgery, chemotherapy, radiation required
Recurrence	Rare	Common

Q119. Systematic desensitization (Definition)

- **Definition:** A behavioral therapy technique to reduce phobias or anxiety by **gradual exposure paired with relaxation**.
- **Process:**
 1. Learn relaxation techniques.
 2. Create anxiety hierarchy (least to most fearful situations).
 3. Gradually expose patient to each level while practicing relaxation.
- **Goal:** Replace fear response with calm, relaxed response.

Q120. Pain behavior classified into four types – write three

1. **Verbal complaints:** Expressing pain through words, moaning, or groaning.
2. **Facial expressions:** Grimacing, frowning, wincing.
3. **Protective behaviors:** Guarding, limping, avoiding movement.

(Fourth type: Social withdrawal or activity reduction.)

Q121. Evidence of cause of type 2 diabetes – enlist 3

1. **Genetic predisposition:** Family history increases risk.
2. **Obesity and sedentary lifestyle:** Excess body fat reduces insulin sensitivity.
3. **Unhealthy diet:** High sugar, refined carbs, and low fiber intake contribute to development.

Q122. Six types of adjustment problems a person experiences – enlist

1. **Emotional problems:** Anxiety, depression, frustration.
2. **Behavioral problems:** Non-adherence to treatment, risk-taking, avoidance.
3. **Cognitive problems:** Difficulty accepting illness, denial, negative thinking.
4. **Social problems:** Isolation, strained relationships, dependency.
5. **Financial problems:** Medical costs, loss of income, treatment burden.
6. **Physical problems:** Fatigue, reduced mobility, pain management challenges.

Q123. Three levels of prevention

1. **Primary prevention:** Preventing disease before it occurs (vaccination, healthy diet, exercise).

2. **Secondary prevention:** Early detection and treatment to prevent progression (screenings, early diagnosis).
3. **Tertiary prevention:** Managing chronic illness to reduce complications (rehabilitation, therapy, lifestyle modification).

Q124. Name two types of coping

1. **Problem-focused coping:** Actively addressing the stressor or illness (e.g., following medical advice, planning, lifestyle changes).
2. **Emotion-focused coping:** Managing emotional response (e.g., meditation, relaxation, social support).

Q125. Three benefits of exercise for weight control

1. **Burns calories:** Creates energy deficit for weight loss.
2. **Increases metabolism and muscle mass:** Maintains lean body weight and improves energy expenditure.
3. **Regulates appetite and reduces stress-related overeating:** Improves hormonal balance and prevents weight regain.

Q126. Causes of asthma

1. **Genetic factors:** Family history of asthma or allergies increases risk.
2. **Allergens:** Dust mites, pollen, pet dander, mold.
3. **Air pollution:** Smoke, chemicals, vehicle emissions.
4. **Respiratory infections:** Viral infections in childhood can trigger asthma.
5. **Exercise-induced:** Physical activity may trigger symptoms in some individuals.
6. **Stress and emotional factors:** Stress can worsen asthma attacks.

Q127. Psychological aspects of stroke

- **Emotional reactions:** Anxiety, depression, frustration, anger.
- **Cognitive effects:** Memory problems, attention deficits, impaired decision-making.
- **Behavioral changes:** Reduced motivation, withdrawal from social activities, non-adherence to therapy.
- **Social impact:** Dependency on family, change in social roles.
- **Interventions:** Counseling, cognitive rehabilitation, support groups, family education, stress management.

Q128. Reinforcement is used for health-related behavior among people

- **Definition:** Using rewards or positive consequences to encourage healthy behavior.
- **Examples in health behavior:**
 1. **Exercise adherence:** Rewarding consistency with praise, social recognition, or incentives.

2. **Medication adherence:** Reminders, feedback, or small incentives to follow prescription.
 3. **Dietary compliance:** Rewarding children for eating healthy foods.
- **Effect:** Reinforcement strengthens desired behaviors and increases motivation for maintaining healthy habits.

Q129. Children copy bad behavior more than good behavior – Discuss

- **Agree/Observation:** Children often imitate **salient, attention-grabbing, or rewarded behaviors**, which may include negative actions.
- **Reasons:**
 1. **Observational learning (Bandura):** Children imitate models (parents, peers, media) they see frequently.
 2. **Immediate reinforcement:** Bad behavior often has visible or fun consequences that attract imitation.
 3. **Cognitive bias:** Children may focus more on dramatic actions rather than subtle positive behaviors.
- **Mitigation:** Encourage positive role models, praise good behavior, reduce exposure to negative behavior, and educate about consequences.

Q130. Note on pain rating scales

- **Definition:** Pain rating scales are tools used to **measure intensity, quality, and impact of pain** in patients.
- **Purpose:** Helps healthcare professionals monitor pain, evaluate treatment effectiveness, and guide interventions.
- **Common scales:**
 1. **Visual Analog Scale (VAS):** A 10-cm line labeled “no pain” to “worst pain”; patient marks intensity.
 2. **Numeric Rating Scale (NRS):** Patient rates pain from 0 (no pain) to 10 (worst pain).
 3. **Faces Pain Scale (FPS):** For children or patients with communication difficulties; select a face representing pain.
 4. **McGill Pain Questionnaire:** Assesses sensory, affective, and evaluative aspects of pain.
 5. **Behavioral Observation:** Observation of protective behaviors, grimacing, limping, or reduced activity.

Q131. Three benefits of exercise

1. **Physical health improvement:** Maintains cardiovascular fitness, strengthens muscles and bones, and aids weight management.
2. **Mental health enhancement:** Reduces stress, anxiety, and depression; improves mood and cognitive function.
3. **Chronic disease prevention:** Lowers risk of diabetes, hypertension, obesity, and certain cancers.

Q132. Scenario – Patient with brain tumor

- **Scenario context:** Patient diagnosed with a brain tumor experiences **physical, cognitive, and emotional challenges**.
- **Psychological aspects:** Anxiety, depression, frustration, fear of surgery or progression, loss of independence.
- **Interventions:**
 1. **Medical treatment adherence:** Surgery, chemotherapy, radiotherapy as advised.
 2. **Psychological support:** Counseling, cognitive-behavioral therapy, relaxation techniques.
 3. **Social support:** Family involvement, support groups, peer support.
 4. **Rehabilitation:** Occupational and physical therapy to maintain daily functioning.
 5. **Lifestyle modifications:** Balanced diet, gentle exercise if approved, adequate rest, and stress management.

Q133. Define health psychology and its scope

- **Definition:** Health psychology is the study of how **psychological, behavioral, and social factors influence physical health and illness**.
- **Scope:**
 1. Understanding how behavior affects health (exercise, diet, smoking).
 2. Studying stress and coping in illness.
 3. Promoting preventive health behaviors.
 4. Supporting chronic illness management and rehabilitation.

Q134. Three cognitive techniques to measure pain

1. **Self-report questionnaires** – Patient describes intensity, duration, and type of pain.
 2. **Pain diaries** – Recording daily pain experience and triggers.
 3. **Visual Analog Scale (VAS) or Numeric Rating Scale (NRS)** – Quantitative assessment of pain level.
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Q135. Three levels of prevention in health psychology

1. **Primary prevention:** Preventing illness before it occurs (vaccinations, exercise, healthy diet).
2. **Secondary prevention:** Early detection to reduce progression (screening, regular check-ups).
3. **Tertiary prevention:** Reducing complications or improving quality of life after illness (rehabilitation, therapy).

Q136. Two types of coping strategies

1. **Problem-focused coping:** Directly managing or solving the stressor (e.g., medication adherence, lifestyle changes).
2. **Emotion-focused coping:** Managing emotional response (e.g., relaxation, social support, meditation).

Q137. Role of social support in chronic illness

- **Definition:** Emotional, informational, and practical assistance provided by family, friends, or community.
- **Importance:**
 1. Reduces stress and anxiety.
 2. Improves adherence to treatment.
 3. Enhances recovery and coping.
 4. Provides emotional and instrumental support (daily activities, counseling).

Q138. Difference between benign and malignant tumors

Feature	Benign	Malignant
Growth	Slow	Rapid
Spread	Localized	Can metastasize
Life risk	Usually not life-threatening	Potentially fatal
Treatment	Often surgical removal	Surgery, chemo, radiotherapy required
Recurrence	Rare	Common

Q139. Systematic desensitization – Definition and steps

- **Definition:** Behavioral therapy to reduce anxiety by gradually exposing patient to fear while practicing relaxation.
- **Steps:**
 1. Learn relaxation techniques.
 2. Build anxiety hierarchy (least to most fearful).
 3. Gradually expose patient while practicing relaxation.
 4. Replace fear response with calm response.

Q140. Scenario-based question – Patient with chronic illness

- **Scenario:** A patient with **diabetes and hypertension** struggles with diet, exercise, and medication adherence.
- **Questions to answer:**
 1. Identify illness-related behaviors.
 2. Suggest coping strategies (problem-focused & emotion-focused).
 3. Recommend social support interventions.
 4. Highlight importance of lifestyle modification.

